

of therapy (LOT) (range, 2–11); 55% had undergone at least one autologous stem cell transplantation. All were triple-class exposed; 50% were penta-exposed, and 34% were penta-refractory. All patients were refractory to their last LOT, with no prior exposure to BCMA-directed therapies. At a median follow-up of 10 months (range: 0.25–29.5), the ORR was 85%, with 79% achieving at least a VGPR. The median time to first and best response was 35 (range, 6–126) and 97 days (range, 12–363), respectively. Median progression-free survival (PFS) was over 2 years, but less than 6 months for patients with EMD. At the time of analysis, 15 patients (28%) had died, predominantly due to progressive disease. Median overall survival (OS) was not reached. All patients experienced at least one AE, with grade ≥ 3 events in 77%. CRS occurred in 50% of them with only one grade ≥ 3 , all during step-up dosing. ICANS was observed in 7 patients (15%) with grade 3 in 1 case. Cytopenias were common: anemia (89%/39% grade ≥ 3), neutropenia (68%/43%), thrombocytopenia (60%/26%), and lymphopenia (87%/70%). Infectious episodes occurred in 32 patients (60%), with grade ≥ 3 infections in 37%. Immunoglobulin substitution was administered to 89% of patients. **Conclusions:** Our real-world data demonstrate that teclistamab achieves comparable efficacy and safety outcomes to those reported in pivotal trials and other real-world studies. The toxicity profile was consistent with previous reports. These findings also underscore the urgent need for novel strategies to improve outcomes in patients with EMD.

PA-513

Clinical Features and Management of GPRC5D related AE in Chinese Relapsed/Refractory Multiple Myeloma Patients Treated with Talquetamab: MonumentAL-1 China Cohort Experience

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Introduction: Talquetamab (tal) is the first and only approved BsAb targeting novel antigen, GPRC5D. Tal has shown overall response rates of >70% in patients(pts) with relapsed/refractory multiple myeloma (RRMM) in the MonumentAL-1 study. Efficacy and most safety data of China cohort had shown consistency with

global cohort, but taste-related events were lower (25.0–41.1%) compared with global cohorts (71.0–72.0%). Here we report the clinical management of GPRC5D related AEs of MonumentAL-1 China cohort. **Methods:** MonumentAL-1 is a phase 1/2, open-label, multicenter study of tal monotherapy in RRMM pts. Phase 1 identified 2 RP2Ds, 0.4 mg/kg QW and 0.8 mg/kg Q2W, which were further assessed in phase 2. Chinese pts were enrolled in phase 2. CRS and ICANS were graded by ASTCT criteria; all other AEs were graded by CTCAE v4.03. Response was assessed per IMWG 2016 criteria. **Results:** 29 pts were enrolled in 0.4 mg/kg QW cohort and 12 pts were enrolled in 0.8 mg/kg Q2W cohort. Updated median follow-up time was 16.3 mo and 13.9 mo respectively ORR was 69% and 67%, $\geq$ VGPR rate was 59% and 58%, $\geq$ CR rate was 38% and 58%. Median DOR was 15.7 mo and not reached. Median PFS was 8.3 and 10.7 mo, 12-month OS rate was 82.2% and 81.8% respectively. GPRC5D “on-target, off-tumor” AE, included oral, skin and nail toxicities. The incidence of dysgeusia was 41% (12/29) in QW cohort and 25% (3/12) in Q2W cohort, median onset time was 15–17 days, median duration was 160–231 days, only 2 pts reported concurrent decreased appetite, and 4 pts reported concurrent $\geq$ 10% weight decrease. 50%–58% dysgeusia was totally resolved at data-cut date. Con-medication for dysgeusia including dexamethasone acetate, prednisone, olanzapine, riboflavin and Chinese herbal. Only 1 pts in Q2W cohort reported dry mouth. No pts had dose modification due to dysgeusia and dry mouth. Skin AEs included rash and non-rash AEs. The incidence of rash was 38% and 25% respectively, with median onset time of 9–25 days and median duration of 17–23 days. Only 1 pts experienced grade 3 rash. Incidence of non-rash skin AEs were 52% and 42%, which included skin exfoliation, dry skin, and pruritus. Only 1 pts reported grade 3 event. Median onset time was 19–62 days, with a median duration of 28–40 days. Supportive care included topical moisturizers, topical corticosteroids, oral corticosteroids, oral anti-allergic medication and Chinese herbals. 79–100% rash were resolved, and 68–100% non-rash skin AEs were resolved. No event led to dose modification. The incidence of nail disorders was 14% and 33% respectively, all grade 1/2, none lead to dose modification. Median onset time was 39–45 days, median duration of 223–301 days. Only 2 pts took topical paraffin and Vitamin E. **Conclusions:** Tal demonstrates deep responses in heavily pre-treated Chinese RRMM patients, with a clinically manageable safety profile. Clinical features of GPRC5D related AEs were similar to global cohort. No dose modifications were required to manage these GPRC5D related AEs in China cohort.

PA-514

Real-World Treatment Patterns, Effectiveness, and Safety of Daratumumab-based Regimens in Chinese Patients with Multiple Myeloma: Final Analysis of the MMY4032 Study

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